NC MEDICAID BEHAVIORAL HEALTH

Operations & Compliance Plan

Progress report printed October 22, 2026

Service lines	CCP 8G Peer Support Services
Footprint	No counties selected
This printout	Entire plan

0%	0 of 27	0 of 103	31 weeks
CHECKLIST COMPLETE	STEPS COMPLETE	ITEMS CHECKED	PLANNED SCHEDULE

PHASE	STEPS	PROGRESS
Phase 1: Business & Legal Foundation	0 of 8 steps	0%
Phase 2: DHSR Licensure (10A NCAC 27G)	0 of 5 steps	0%
Phase 3: NCTracks & State Integration	0 of 4 steps	0%
Phase 4: Tailored Plan Contracting	0 of 1 steps	0%
Phase 5: Launch & Ongoing Compliance	0 of 3 steps	0%
Phase 6: H.R. 1 Medicaid Compliance	0 of 6 steps	0%

Estimated statutory fees: $1,093–$1,440, plus $73–$113 per staff member for screening. Excludes buildout, lease and salaries.

☐ To do ☑ Done **Status:** NOT STARTED, IN PROGRESS, COMPLETED or BLOCKED shown beside each step

Phase 1: Business & Legal Foundation

0 of 8 steps complete

Step 1 Entity Structure, Ownership Ledger & Labor Classification

NOT STARTED

Establish your legal entity (LLC, PC, or 501c3) with documented governance. Configure non-exempt wage classification under the NC Wage & Hour Act (N.C.G.S. § 95-25) ensuring Peer Support Specialists (CPSS) are paid for note documentation and inter-site travel.

Owner: Owner / Healthcare Attorney & CPA **Time:** 1–3 weeks
Cost: $125 LLC or corporation ($60 nonprofit) + $15 Certificate of Existence

Depends on: None — Initial foundation **Where:** NC Secretary of State (SOS), https://www.sosnc.gov/

- ☐ File Articles of Organization/Incorporation with NC Secretary of State
- ☐ Obtain certified Certificate of Existence from NC SOS
- ☐ Execute corporate operating agreement naming governing body oversight (10A NCAC 27G .0201)
- ☐ Create 5%+ ownership disclosure ledger required for DHSR & 42 CFR § 455.104
- ☐ Establish hourly non-exempt compensation policy for CPSS covering travel and service note writing

Step 2 Federal Tax Identification Number (EIN)

NOT STARTED

Obtain an Employer Identification Number from the IRS for payroll, federal healthcare tracking, and NPI registration.

Owner: Owner / CFO **Time:** Immediate (online) **Cost:** $0 (Free)

Depends on: Step 1 (Entity registration)
Where: IRS EIN Assistant, https://www.irs.gov/businesses/small-businesses-self-employed/apply-for-an-employer-identification-number-ein-online

- [ ] Complete online IRS Form SS-4
- [ ] Download and archive official EIN Confirmation Letter (Notice CP 575)
- [ ] Verify legal entity name matches exactly with NC SOS registration

Step 3 National Provider Identifier (NPI Type 2 Organization)

NOT STARTED

Acquire organizational Type 2 NPI from NPPES. Necessary before initiating NCTracks or DHSR facility linkages.

Owner: Office Administrator / Compliance Officer **Time:** 1–3 business days **Cost:** $0 (Free)

Depends on: Step 1 & Step 2 (Entity name & EIN) **Where:** NPPES NPI Registry, https://nppes.cms.hhs.gov/

- [ ] Register Authorized Official in NPPES portal
- [ ] Apply for Type 2 Organizational NPI
- [ ] Assign behavioral health / SUD organization primary taxonomy

Step 4 Facility Site Selection & Municipal Zoning Approval

NOT STARTED

Secure commercial day or outpatient premises sized for the client census. Get written zoning approval for an outpatient substance use day program; DHSR asks for the original zoning letter with the application. DHSR lists Group B Business Occupancy for adult SAIOP and SACOT, and adolescent day programs cannot be in a Business Occupancy. A peer-support-only site may not trigger DHSR zoning paperwork, so confirm with your plan.

Owner: Facility Director **Time:** 3–6 weeks
Cost: Varies by lease. Local zoning letter fees vary by city or county (Durham charges $28 to $58).

Depends on: Step 1 (Entity formed)
Where: County and city planning offices (the NCACC link lists counties only), https://www.ncacc.org/county-websites/

- [ ] Execute lease or purchase agreement with confidentiality/counseling room partitions
- [ ] Request formal Zoning Determination Letter from city/county planning department
- [ ] Verify physical site meets ADA accessibility requirements

Step 5 Commercial, General & Professional Liability Insurance

NOT STARTED

Bind commercial general liability, professional liability and workers compensation coverage. Limits come from your LME/MCO contracts (for example Alliance asks for $1M per occurrence and $3M aggregate), not from the DHSR application, so get each plan insurance requirements in writing.

Owner: Owner / Risk Manager **Time:** 1–2 weeks **Cost:** $1,500 – $4,500 / year (estimate, not verified)

Depends on: Step 4 (Site lease secured) **Where:** Commercial Healthcare Insurance Broker, https://www.ncdoi.gov/

- [ ] Procure $1M / $3M professional malpractice liability policy
- [ ] Procure commercial general liability & property coverage
- [ ] Procure NC statutory Workers Compensation insurance
- [ ] Ensure certificates show your agency as the named insured and the LME/MCO as certificate holder (additional insured if the plan requires it), with primary and non-contributory wording where available

Step 6 Dedicated Business Bank Account & EFT Setup

NOT STARTED

Open a dedicated commercial bank account in the exact legal name of the entity for Medicaid electronic fund transfers (EFT). NCTracks takes EFT details keyed into the enrollment application; a voided check or bank letter is mainly requested by LME/MCOs.

Owner: Owner / Treasurer **Time:** 1–2 business days **Cost:** $0 – $100 initial deposit (estimate, not verified)

Depends on: Step 1, 2, 3 (Entity, EIN, NPI) **Where:** Commercial Banking Institution, https://www.fdic.gov/

- ☐ Establish a business checking account in the exact legal entity name and keep a voided check for LME/MCO EFT forms
- ☐ Obtain a bank letter verifying routing and account numbers for LME/MCO EFT forms (NCTracks takes EFT details online)
- ☐ Establish segregated payroll holding ledger

Step 7 Governing Body Appointment & Key Clinical Leadership

NOT STARTED

Appoint the Facility Director named on the DHSR application and a Clinical Director. SAIOP and SACOT programs must be directed by an LCAS or CCS (on site at least 50% of SAIOP hours and 90% of SACOT hours, 10A NCAC 27G .4402 and .4502). Qualified Professional standards (27G .0104) apply to peer support program supervision and direct care staff.

Owner: Governing Board / Owner **Time:** 1–3 weeks **Cost:** $0 (Internal governance)

Depends on: Step 1 (Corporate governance) **Where:** 10A NCAC 27G .0104 Definitions, https://www.oah.nc.gov/

- ☐ Document Board of Directors / Advisory Committee minutes
- ☐ Designate the Facility Director named on the DHSR application and delegate management authority in writing (the license is issued to the entity, not the director)
- ☐ Appoint a Clinical Director who is an LCAS or CCS for SAIOP and SACOT, or a full-time QP for peer support (10A NCAC 27G .0104)
- ☐ Draft organizational chart showing lines of clinical and administrative supervision

Step 8 LME/MCO Network Need Inquiry (a 27G .0406 Letter of Support is for residential facilities)

NOT STARTED

Ask each Tailored Plan that covers your county whether it has a network need for your services. DHSR does not require an LME/MCO Letter of Support (10A NCAC 27G .0406) for day programs; that rule applies to 24-hour residential facilities. Peer support has been a closed-network service since July 7, 2026, so plans may decline new applicants. Vaya Health and Partners merged on Oct 1, 2026.

Owner: Facility Director **Time:** Varies by plan (Alliance answers provider applications in 5 business days)
Cost: $0 (Administrative)

Depends on: Step 4 & 7 (Facility site & leadership)
Where: NC DHHS LME/MCO directory, https://www.ncdhhs.gov/providers/lme-mco-directory

- ☐ Submit a program summary and county location to the Tailored Plan that covers your county and ask whether it is accepting new providers
- ☐ List the services you plan to offer (Peer Support, SAIOP, SACOT) and ask whether each is a current network need in your county
- ☐ Get the plan written network-need or contract-status response (a .0406 Letter of Support is only for residential facilities)

Phase 2: DHSR Licensure (10A NCAC 27G)

0 of 5 steps complete

Step 9 Statutory Policies & Procedures Engineering

NOT STARTED

Draft the policy and procedure manual from the DHSR Policy and Procedure Workbook, cross-referenced to 10A NCAC 27G, the DHHS Records Management and Documentation Manual (RMDM, effective July 8, 2025), 42 CFR Part 2 (compliance date Feb 16, 2026) and IRIS incident reporting. DHSR says not to send policies with the application; your consultant reviews them later.

Owner: Clinical Director / Compliance Consultant **Time:** 3–6 weeks **Cost:** $0 – $3,500 (consultant optional)

Depends on: Step 7 & 8 (Leadership named, Letter of Support in progress)
Where: DHSR Policy and Procedure Workbook, https://info.ncdhhs.gov/dhsr/mhlcs/pdf/PolicyProcedure-Workbook.pdf

- ☐ Draft governance, client rights and grievance procedures (10A NCAC 27G .0201, 27D and 27E)
- ☐ Integrate RMDM service note requirements cited in CCP 8G, 8A-12 and 8A-13
- ☐ Incorporate 42 CFR Part 2 consent, notice and disclosure rules (compliance date Feb 16, 2026)
- ☐ Establish Incident Response Improvement System (IRIS) 72-hour reporting workflow (.0600)
- ☐ Draft Fraud, Waste & Abuse (FWA) / False Claims Act compliance plan

Step 10 Core Clinical & Peer Support Staff Recruitment

NOT STARTED

Recruit and hire staff who meet the rules and the CCPs: an LCAS or CCS clinical supervisor (a Clinical Supervisor Intern is also allowed) for SAIOP and SACOT, Licensed or Certified Clinical Staff for groups, and NC Certified Peer Support Specialists (CPSS) supervised by a QP at 1:8.

Owner: Clinical Director / HR **Time:** 4–8 weeks **Cost:** Recruitment costs / Staff payroll

Depends on: Step 7 & 9 (P&P and leadership established)
Where: NCASPPB (LCAS and CCS credentials) and the NC CPSS Program, https://www.ncsappb.org/credentialing/licensed-clinical-addiction-specialist/

- ☐ Hire LCAS (Licensed Clinical Addictions Specialist) or CCS for SUD supervision
- ☐ Verify SACOT staffing: at least 1 clinical staff for every 10 or fewer beneficiaries in group counseling (CCP 8A-13 section 6.2)
- ☐ Verify Peer Support staffing: 1 QP per 8 CPSS, and CPSS to beneficiary ratios of 1:15 individual and 1:12 group (CCP 8G section 6.2.1)
- ☐ Execute signed employment agreements with defined job descriptions

Step 11 Mandatory Pre-Hire Screenings (HCPR & Exclusion Checks)

NOT STARTED

Perform pre-employment checks: the NC Health Care Personnel Registry (HCPR) under N.C.G.S. § 131E-256 and 10A NCAC 27G .0202(b)(4), state and national criminal history checks (G.S. 122C-80), and exclusion screening against the OIG LEIE, SAM.gov and the NC Medicaid exclusion list.

Owner: HR Director / Compliance Officer **Time:** 1–2 weeks
Cost: $35 – $75 per candidate (estimate; the SBI state check is capped at $14 by statute and no national check price is published)

Depends on: Step 10 (Candidates identified)
Where: NC Health Care Personnel Registry (HCPR), https://info.ncdhhs.gov/dhsr/hcpr/index.html

- ☐ Execute and file NC Health Care Personnel Registry (HCPR) check PRIOR to hire date
- ☐ Conduct state and national criminal history checks with fingerprints
- ☐ Screen staff and 5%+ owners against OIG LEIE, SAM.gov, and NC Exclusion database
- ☐ Establish recurring monthly OIG exclusion audit calendar

Step 12 CPSS Credential & Lived-Experience Verification

NOT STARTED

Verify each Peer Support staff member holds an active NC CPSS certification from the NC Peer Support Specialist Certification Program (pss.unc.edu). Certification requires 18 months in recovery before training. CCP 8G (amended Sept 1, 2026) requires a roster of CPSS staff with certification and hire dates.

Owner: Clinical Director / HR **Time:** 1–2 weeks **Cost:** $0 (Online verification)

Depends on: Step 10 (CPSS hired) **Where:** UNC NCCPSS Program Office, https://pss.unc.edu/

- ☐ Verify active credential on NCCPSS registry
- ☐ File proof of training: Foundations of Peer Support (60 hours on Track A) or the approved 50-hour course (Track B), plus the 20-hour prerequisite training (3 hours of ethics and 17 related hours)
- ☐ Verify lived experience attestation in HR record
- ☐ Set 2-year certification renewal reminder

Step 13 Local Fire Marshal & Sanitation Facility Clearances

NOT STARTED

Schedule and complete the local fire safety inspection and any county health department sanitation review. Fire and safety compliance is annual for licensure (10A NCAC 27G .0402(d)(3)); DHSR requires a sanitation report for day programs only if you serve food.

Owner: Facility Director **Time:** 2–4 weeks **Cost:** $75 – $250 inspection fees (estimate; varies by jurisdiction, not verified)

Depends on: Step 4 (Site secured)
Where: Local Municipal Fire Marshal & County Health Dept, https://www.ncdhhs.gov/divisions/public-health/county-health-departments

- ☐ Pass annual Fire Marshal inspection with stamped approval certificate
- ☐ Get local health department approval of water and sewage (10A NCAC 27G .0303) and a sanitation report if you serve any food
- ☐ Ask the fire code official for the maximum occupant load of each room and post signs if the official requires them

Phase 3: NCTracks & State Integration

0 of 4 steps complete

Step 17 NC HealthConnex (HIE) Participation Agreement & Integration

NOT STARTED

Complete the NC HealthConnex participation agreement (a PDF emailed to hiea@nc.gov, not signed online; allow 6 to 8 weeks) and set up an EHR that can send HL7 version 2.0 or higher messages. Records covered by 42 CFR Part 2 are exempt from submission under G.S. 90-414.4(c).

Owner: EHR Administrator / Compliance Officer **Time:** 6–8 weeks **Cost:** $0 state fee (EHR interface fee varies)

Depends on: Step 1 (Entity formed); the DHSR license is not required first
Where: NC HealthConnex / NC HIEA, https://hiea.nc.gov/

- ☐ Complete and email the NC HealthConnex participation agreement to HIEA (it is not signed online)
- ☐ Select an EHR that can send HL7 v2.0 or higher messages to NC HealthConnex and can filter 42 CFR Part 2 records
- ☐ File written confirmation of your connection status from HIEA (no source found requires proof before billing; confirm with your plans)

Step 18 NCTracks Organization Provider Enrollment Application

NOT STARTED

Submit the Organization Enrollment Application in NCTracks. The Office Administrator must be an owner or managing employee with an NCID. Select taxonomy 251S00000X and each service you offer. Finish the NC Medicaid New Provider Training within 15 calendar days of submitting.

Owner: Office Administrator **Time:** 1–2 weeks to submit
Cost: $100 NC state fee + $750 federal application fee (2026 amount; the 2027 amount applies from Jan 1, 2027)

Depends on: Step 3 (NPI) and, for SAIOP and SACOT, Step 16 (DHSR license)
Where: NCTracks Provider Portal, https://www.nctracks.nc.gov/content/public/providers/provider-enrollment.html

- ☐ Obtain NCID for designated Office Administrator
- ☐ Complete NCTracks Organization Enrollment Application
- ☐ Select taxonomy 251S00000X and add each service (Peer Support, SAIOP, SACOT) as the Provider Permission Matrix lists them
- ☐ Sign the W-9 attestation, enter EFT details online, and upload only the documents the application asks for (NCTracks verifies the DHSR license)

Step 19 Moderate/High Risk Fingerprinting & Site Screening

NOT STARTED

Complete fingerprint-based criminal history checks for owners with 5% or more ownership (managing employees are not required to be fingerprinted) under N.C.G.S. § 108C-3, and pass the NCTracks site visit. The Provider Permission Matrix lists peer support, SAIOP and SACOT as high risk. Per a Sept 14, 2026 NC Medicaid bulletin, the site visit and fingerprinting are waived only if you hold active national accreditation for the same location and service.

Owner: 5%+ Owners / Managing Employees **Time:** 3–5 weeks
Cost: $38 per person (SBI/FBI electronic fingerprinting; NCTracks says the cost varies by location, so confirm)

Depends on: Step 18 (NCTracks notification received)
Where: Local Sheriff's Office & NCTracks Screening, https://www.nctracks.nc.gov/

- ☐ Schedule LiveScan fingerprinting at an approved site after the NCTracks notice arrives, before the deadline in the notice
- ☐ Upload the NCTracks-generated fingerprint Release of Information forms in the portal (mailed forms are not processed)
- ☐ Pass NCTracks physical site visit verification

Step 20 NCTracks Approval & Effective Date Alignment

NOT STARTED

Check the enrollment effective date on the approval letter. If clinicians are enrolled as individual providers (for example an LCAS), the Office Administrator of each individual record affiliates them to your organization; CCS and QP are not NCTracks provider types.

Owner: Office Administrator **Time:** 2–4 weeks **Cost:** $0

Depends on: Step 18 & 19 (Enrollment and screening approved)
Where: NCTracks Provider Record, https://www.nctracks.nc.gov/

- ☐ Verify billing effective date aligns with credentials
- ☐ Have each individually enrolled clinician Office Administrator submit a Manage Change Request to affiliate to your organization NPI (confirm with NC Medicaid whether agency staff must be enrolled)
- ☐ Test electronic claims submission and ERA 835 delivery

Phase 4: Tailored Plan Contracting

0 of 1 steps complete

Step 21 Regional Tailored Plan / LME-MCO Network Participation

NOT STARTED

Apply to and contract with each Tailored Plan that covers your county: Alliance, Trillium or Vaya Partners (Vaya Health and Partners merged on Oct 1, 2026). NCTracks enrollment comes first. Peer support is a closed-network service, so a plan may decline new applicants unless it has a posted need.

Owner: Executive Director / Contracting Officer **Time:** 6–12 weeks **Cost:** $0 (Contracting)

Depends on: Step 18 (NCTracks enrollment active) and, for SAIOP and SACOT, Step 16 (DHSR license)
Where: NC DHHS LME/MCO directory, https://www.ncdhhs.gov/providers/lme-mco-directory

- ☐ Submit network participation applications to applicable regional plans
- ☐ Submit each plan clinician roster or enrollment form with NCTracks records (no plan page found asks for CAQH IDs; confirm with each plan)
- ☐ Establish Electronic Data Interchange (EDI) testing with plan clearinghouses
- ☐ Complete Utilization Management (UM) portal training for prior authorizations

Phase 5: Launch & Ongoing Compliance

0 of 3 steps complete

Step 22 EHR Clinical Setup & Service Note (RMDM) Scrubber

NOT STARTED

Configure EHR clinical note templates that meet the DHHS Records Management and Documentation Manual (RMDM), including service start and end times, 15-minute units (H0038) for Peer Support, and same-episode service exclusion checks.

Owner: Clinical Director & Billing Specialist **Time:** 2–4 weeks **Cost:** $150 – $400 / clinician / month (estimate, not verified)

Depends on: Step 9 & 17 (P&P and HIE interface ready)
Where: EHR vendor (healthit.gov is general guidance, not a vendor list), https://www.healthit.gov/

- ☐ Configure service note templates that meet RMDM requirements, including start and end times
- ☐ Build billing rules that block SAIOP and SACOT in the same episode of care (except admission and discharge days) and block Peer Support at the same time of day as SAIOP or SACOT
- ☐ Configure Peer Support units: 15-minute H0038 units, 24 unmanaged units per episode, then 176 units per 60 days and no more than 88 per 30 days (CCP 8G); do not hard-code daily or weekly caps unless a plan requires them
- ☐ Configure 3-hr daily minimum for SAIOP (H0015, one event per day) and 4-hr daily minimum for SACOT (H2035, one unit per hour)

Step 24 Intake, Assessment, PCP & Authorization Protocol

NOT STARTED

Standardize intake: a comprehensive clinical assessment completed before service, ASAM level-of-care determination, a Service Order signed by a physician, physician assistant, nurse practitioner or licensed psychologist (confirm the signer list and term in each CCP; CCP 8G differs), and tracking of any authorization periods your plan requires. CCP 8A-12 and 8A-13 require no prior approval; CCP 8G authorizes up to 176 units per 60 days.

Owner: Clinical Supervisor / Intake Coordinator **Time:** 1–2 weeks **Cost:** $0 (Clinical workflow)

Depends on: Step 21 & 22 (Network contract and EHR active)
Where: NC Medicaid Person-Centered Plan (PCP) Manual, https://medicaid.ncdhhs.gov/

- ☐ Verify CCA contains required ASAM 6-dimension assessment (Level 2.1 SAIOP / Level 2.5 SACOT)
- ☐ Ensure Service Order is executed prior to or on Day 1 of service
- ☐ Set up automated tracking of Peer Support authorization periods (176 units per 60 days, no more than 88 per 30 days) and confirm with each Tailored Plan whether SAIOP or SACOT need any authorization
- ☐ Execute modernized 42 CFR Part 2 and HIPAA consent documentation at intake

Step 25 National Accreditation Roadmap & NCTracks 5-Yr Reverification

NOT STARTED

Plan for national accreditation within 1 year of enrollment. CCP 8A-12 and 8A-13 say 1 year; the Provider Permission Matrix says 3 years for SAIOP and SACOT and 1 year for peer support, so plan for 1 year until NC Medicaid clarifies. Accepted bodies are COA, The Joint Commission and CARF; the Aug 16, 2026 matrix also lists CQL, which the April bulletin does not. Establish a compliance calendar for annual DHSR renewal, 2-yr CPSS renewal, and 5-yr NCTracks revalidation.

Owner: Quality Director / CEO **Time:** Ongoing (accreditation deadline runs from enrollment; see the instructions)
Cost: $8,000 – $14,000 (estimate, not verified; CARF says the fee depends on surveyors, survey days and travel)

Depends on: Step 20 & 21 (Operational and billing Medicaid)
Where: CARF International Behavioral Health, https://www.carf.org/

- ☐ Select an accepted accreditation body (CARF, COA or The Joint Commission)
- ☐ Run continuous 6-month demonstration period with QI Committee and chart peer reviews
- ☐ Calendar NCTracks 5-year revalidation (first notice 70 days out, reminders at 50, 20 and 5 days; a missed deadline means suspension, then termination)
- ☐ Calendar monthly OIG/SAM exclusion screenings and 2-year CPSS renewals

Phase 6: H.R. 1 Medicaid Compliance

0 of 6 steps complete

Step 26 H.R. 1 Impact Review & Compliance Owner

NOT STARTED

H.R. 1 (Public Law 119-21, signed July 4, 2025) changes Medicaid eligibility, renewals, cost-sharing and program-integrity rules and created the Rural Health Transformation Program. Most of it binds the state and members, so providers mainly adapt. Verified dates: Oct 1, 2026 (many noncitizens lose Medicaid); Jan 1, 2027 (work requirement, six-month renewals for expansion adults, retroactive coverage cut to 1 month for expansion adults); July 1, 2027 (NC raises copays to the federal maximum); Oct 1, 2028 (federal cost-sharing, which excludes mental health and SUD services). NC has told CMS it is unlikely to fully implement the work requirement by Jan 1, 2027.

Owner: Compliance Officer / Owner **Time:** 1–2 weeks **Cost:** $0 (Internal)

Depends on: Step 1 (Entity formed)
Where: H.R. 1 (119th Congress) bill text, https://www.congress.gov/bill/119th-congress/house-bill/1

- ☐ Review H.R. 1 provisions and NC Medicaid H.R. 1 provider guidance that affect client eligibility, billing and documentation
- ☐ Assign H.R. 1 monitoring to your existing compliance officer, with a backup, and record it in governing body minutes
- ☐ Subscribe to NC Medicaid bulletins and each Tailored Plan provider notice list for H.R. 1 updates
- ☐ Build a dated H.R. 1 calendar of effective dates, each with an internal deadline and an owner

Step 27 Eligibility Verification & Six-Month Renewal Readiness

NOT STARTED

Most expansion adults move to six-month eligibility renewals starting Jan 1, 2027, and many noncitizens lose coverage Oct 1, 2026, so more clients will lose coverage. NC Medicaid policy requires you to verify eligibility each time a service is rendered. Build intake and billing controls for that, and help clients keep their coverage without giving legal advice.

Owner: Billing Manager / Intake Lead **Time:** 2–4 weeks **Cost:** $0 (Internal workflow)

Depends on: Step 18 (NCTracks enrollment) & Step 22 (EHR setup)
Where: NCTracks (eligibility verification), https://www.nctracks.nc.gov/

- ☐ Verify Medicaid eligibility and health plan at intake and on every date of service
- ☐ Create a claim-hold and follow-up workflow for clients whose eligibility lapses during treatment
- ☐ Train staff to remind clients about renewal notices and refer them to their county DSS or NC Medicaid for help
- ☐ Track monthly Medicaid coverage lapses and their reasons for quality improvement and governing body review

Step 28 Community Engagement Requirement & Exemption Documentation

NOT STARTED

The work requirement (80 hours a month for expansion adults aged 19 to 64, from Jan 1, 2027) has exemptions, but they are not automatic. People with a substance use disorder or a disabling mental disorder count as medically frail, and people in a qualifying treatment program are excluded, but stable recovery of 5+ years is excluded from the SUD category, the condition must significantly impair compliance, and the program exclusion covers only nonprofit or public programs, so a for-profit SAIOP or SACOT may not qualify. NC has draft policy HR-1, a clinician-signed DSS-8209 attestation and a medically frail code list open for comment Oct 6 to Nov 5, 2026. Providers document; county DSS decides.

Owner: Clinical Director / Compliance Officer **Time:** 3–4 weeks **Cost:** $0 (Internal)

Depends on: Step 26 (Impact review) & Step 24 (Intake and assessment)
Where: NC Medicaid (NC DHHS), https://medicaid.ncdhhs.gov/

- ☐ Confirm NC Medicaid final medical frailty policy and DSS-8209 process; providers document, and county DSS decides
- ☐ Add a client consent form for sharing treatment participation or exemption information, including the 42 CFR Part 2 rules for substance use disorder records
- ☐ Prepare clinicians to complete the NC DSS-8209 Medical Report for SUD treatment participation, SUD, disabling mental disorder or medical frailty exclusions
- ☐ Set up EHR reports showing each client program enrollment dates and active participation to support DSS-8209 and DSS verification requests
- ☐ Train staff on what they may tell clients about the requirement and when to refer them to NC Medicaid or county DSS
- ☐ Review the NC draft medical frailty code list and send comments by Nov 5, 2026 (H0015 and H2035 are on it; H0038 is not)

Step 29 Coverage Changes, Billing & Cost-Sharing Controls

NOT STARTED

Retroactive coverage drops to 1 month for expansion adults and 2 months for others for applications on or after Jan 1, 2027. Many noncitizens lose coverage Oct 1, 2026. Federal cost-sharing starts Oct 1, 2028 and excludes mental health and SUD services; NC raises its copays to the federal maximum on July 1, 2027. Update intake, financial-responsibility and billing rules so you do not deliver services you cannot get paid for or collect amounts you are not allowed to charge.

Owner: Billing Manager / Compliance Officer **Time:** 2–3 weeks **Cost:** $0 (Internal policy work)

Depends on: Step 26 (Impact review) & Step 20 (NCTracks approval)
Where: NC Medicaid (NC DHHS), https://medicaid.ncdhhs.gov/

- ☐ Update the intake and billing policy for shorter retroactive coverage: confirm eligibility before services rather than relying on back-dated approval
- ☐ Write a financial responsibility policy with client notices (including the Good Faith Estimate for uninsured and self-pay clients) and a sliding-fee option for people who lose or do not qualify for Medicaid
- ☐ Confirm with each Tailored Plan whether NC copays (from July 1, 2027) apply to peer support, SAIOP and SACOT; federal cost-sharing from Oct 1, 2028 excludes mental health and SUD services
- ☐ Update billing system edits for new eligibility categories and immigration-status changes, and test claims before launch

Step 30 Program Integrity, Provider Screening & Funding Risk

NOT STARTED

H.R. 1 adds death-record checks of provider enrollment (a state duty from Jan 1, 2028) and limits provider taxes and state directed payments, which can pressure Medicaid funding. Providers have no new filing, but monthly exclusion screening and accurate NCTracks records are already required. Plan for rate or volume changes.

Owner: Compliance Officer / CFO **Time:** 3–4 weeks **Cost:** $0 (Internal)

Depends on: Step 20 (NCTracks approval) & Step 25 (Compliance calendar)
Where: OIG Exclusions Database (LEIE), https://oig.hhs.gov/exclusions/

- ☐ Add a monthly screen of every owner, manager and rendering provider against the OIG exclusion list, SAM.gov and the NC Medicaid exclusion list (a participation agreement requirement)
- ☐ Calendar NCTracks updates for ownership, address, license, NPI and taxonomy changes, with the reporting deadline for each
- ☐ Build a reimbursement-risk budget that shows cash needs if Medicaid volume or rates fall 10 to 20 percent
- ☐ Set a cash reserve target and a review date tied to NC Medicaid rate and Tailored Plan contract notices

Step 31 Rural Health Transformation (NC ROOTS) Funding (optional)

NOT STARTED

H.R. 1 funds $10 billion a year for FY2026 to FY2030 for state rural health plans; NC runs it as NC ROOTS (year 1 award about $213 million) through five regional hub leads. This is optional funding, not a provider obligation. The Rural Health Innovation Fund lists behavioral health providers as eligible, with the full application due Nov 16, 2026 (RHIF@nc.gov, 919-754-6733). CMS limits direct payments to providers to 15% of a state award.

Owner: Owner / Grants Lead **Time:** 4–8 weeks **Cost:** $0 (Internal)

Depends on: Step 1 (Entity formed) & county selection in Setup
Where: NC Department of Health and Human Services, https://www.ncdhhs.gov/

- ☐ Find NC rural health transformation (NC ROOTS) announcements, funding notices and eligibility rules from NC DHHS
- ☐ Contact the hub lead for each of your counties (see Regional contacts) and ask how providers join or apply
- ☐ Prepare a one-page capability statement with services, counties served, peer workforce and expected outcomes
- ☐ Calendar application deadlines, reporting requirements and any data or technology conditions

Progress, statuses and notes reflect the dashboard on October 22, 2026. Fee amounts, phone numbers and requirements are planning aids: confirm them with the issuing agency before relying on them.